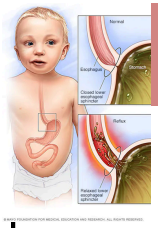
GI

GERD

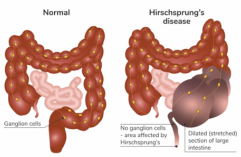


- Reflux causes troublesome symptoms or poor growth.
- Smaller appropriate feeds; avoid overfeeding; burp.
- Upright while awake after feeds.
- Supine on flat surface for sleep; no wedges.
- Medicines only when indicated.

Dehydration

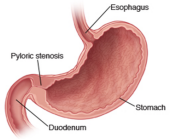
- Less urine/tears, dry mouth, tachycardia, lethargy.
- Assess weight change, perfusion and electrolytes.
- Oral rehydration for suitable mild/moderate cases.
- Severe dehydration/shock: prescribed isotonic IV fluids; reassess.

Hirschsprung's Disease



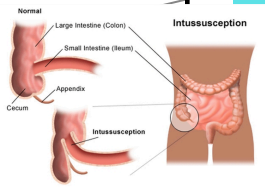
- Absent intestinal ganglion cells → functional obstruction.
- Delayed meconium, distention, bilious vomiting.
- Rectal biopsy confirms.
- Surgical pull-through; watch fever/explosive diarrhea for enterocolitis.

Pyloric Stenosis



- Projectile nonbilious vomiting; hungry after feeds.
- Olive-like mass; dehydration/weight loss.
- Ultrasound; correct fluids/electrolytes before pyloromyotomy.

Intussusception



- Bowel telescopes: episodic pain, knees drawn up, vomiting.
- Blood/mucus stool is a late clue.
- NPO, fluids, ultrasound; air/contrast enema if appropriate.
- Perforation/shock → surgical care.

Cleft Lip/Palate



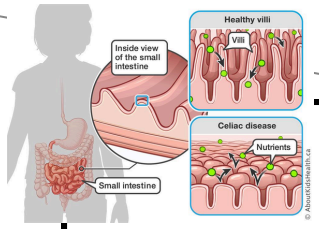
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- Special feeding method; upright positioning and frequent burping.
- After repair: protect incision, prescribed feeding, pain control.
- Avoid unapproved oral instruments; follow surgeon instructions.

Infectious GI Disorders

- Vomiting/diarrhea: assess hydration, electrolytes and stool history.
- Oral rehydration and hygiene; precautions per cause.
- Bloody stool, bilious emesis or poor perfusion needs urgent review.

Inflammatory GI Disorders



Appendicitis



- Pain often shifts from periumbilical to right lower quadrant; fever, anorexia, nausea.
- NPO; IV fluids, analgesia and antibiotics as ordered; surgical evaluation.
- Avoid laxatives/enemas and abdominal heat.
- Worsening diffuse pain/rigidity or sepsis may indicate perforation.

Celiac Disease

- Immune reaction to gluten damages small-intestinal villi.
- Diarrhea, bloating, poor growth; iron deficiency may occur.
- Serology ± biopsy while consuming gluten as directed.
- Lifelong gluten-free diet: avoid wheat, barley, rye.
- Dietitian support; prevent cross-contact.

Nursing Considerations (GI)

- Hydration/perfusion first: weight, urine, vitals, mucosa.
- Track emesis/stool color, frequency and volume.
- Bilious vomiting or rigid/distended abdomen: urgent review.
- Prescribed diet/NPO, fluids and medicines.
- Family teaching: hygiene, feeding plan and return precautions.

Other GI Conditions

Constipation



- Hard/painful or infrequent stools; withholding.
- Age-appropriate fiber/fluids; routine toilet sitting.
- Stool softener/disimpaction per plan; no blame.

Meconium Plug

- Colonic stool plug → delayed stool/distention. Contrast enema may diagnose and relieve.

Meconium Ileus

- Thick meconium blocks ileum; linked to CF. Bilious emesis/distention → urgent care.

Imperforate Anus

- Absent/misplaced anal opening; no stool. NPO, surgical assessment; no rectal instrumentation.